MEZO NETWORK: FACILITY ASSESSMENT CRITERIA

Version 2026-10-v1. Applies to every facility that hosts Mezo patients, and is the condition for signing the Mezo Surgery Facility Agreement.

HOW A FACILITY JOINS

A facility joins the Mezo Network in three steps.

1. **Minimum requirements.** Every item in Part A must be met, with evidence, before anything else is scored. These are pass or fail. A facility that misses one does not join, however well it does elsewhere.
2. **Scored assessment.** Part B scores the facility across nine domains. The score, and the specific items in Part C, decide its **Surgical Tier**: the class of surgery it may host.
3. **Decision and agreement.** A written report within ten (10) working days of the site visit gives one of three outcomes. **Approved** at a named Tier, after which the Facility Agreement is signed for that Tier and that list of procedures. **Conditionally approved**, where every minimum is met but scored items fall short: the facility may sign for a lower Tier now, with a corrective plan of no more than ninety (90) days. **Not approved**, with the reasons and the earliest date to reapply.

A facility may host consultations under the Facility Partnership and Licence Agreement on the minimum requirements alone. Surgery needs a Tier.

PART A: MINIMUM REQUIREMENTS TO JOIN

Registration and governance

A1. Current registration with the state regulator for private health establishments (HEFAMAA in Lagos, and the equivalent body in the FCT and other states), for the services the facility will provide.

A2. A named medical director with full, current MDCN registration and practising licence, who is medical director of record for this facility.

A3. Current registration of the pharmacy with the Pharmacists Council of Nigeria and of the laboratory with the Medical Laboratory Science Council of Nigeria, where these are on site. Where they are not, a written arrangement with registered providers.

A4. Where X-ray or a C-arm is used, a current licence from the Nigerian Nuclear Regulatory Authority.

A5. Public liability insurance, and evidence that every clinician working there holds professional indemnity cover.

Safety in every procedure

A6. The WHO Surgical Safety Checklist in use for every operation, with completed checklists retained in the record.

A7. Continuous pulse oximetry, blood pressure monitoring and ECG for every patient under anaesthesia or sedation, and capnography for every patient with an airway device, in line with the WHO-WFSA International Standards for a Safe Practice of Anaesthesia.

A8. A trained anaesthesia provider present throughout every general or regional anaesthetic.

A9. A resuscitation trolley with a working defibrillator, airway equipment and emergency drugs, checked and signed daily, in or beside every theatre and recovery area.

A10. Piped or cylinder oxygen with a documented reserve, and suction, at every theatre and recovery bed.

A11. An uninterrupted power arrangement for theatres and recovery: generator with automatic changeover, plus battery backup for monitors and lights.

Infection prevention

A12. Instrument sterilisation by a validated steam autoclave, with a cycle log and routine chemical and biological indicator testing.

A13. Clinical waste segregated and collected by a licensed contractor.

Escalation

A14. A written plan for the deteriorating patient, and for transfer to intensive care: on site, or by a written agreement with a named hospital and an ambulance arrangement.

A15. Access to screened, cross-matched blood, on site or from a named blood bank within an agreed time.

Records and data

A16. A clinical record for every patient with operative and anaesthetic notes, and a process for consent that records the risks discussed.

A17. A named person responsible for patient data, and processing in line with the Nigeria Data Protection Act 2023.

Fire and premises

A18. A current fire safety certificate, and signed-off emergency exits and fire equipment.

PART B: SCORED ASSESSMENT

Each item is scored **0** (not in place), **1** (in place, not consistent or not documented) or **2** (in place, documented and consistently followed). Evidence is checked on site, from documents, by tracing five recent surgical patients through their records, and by talking to staff.

Domain	What is assessed	Items
B1. Leadership and governance	Clinical governance meeting, incident reporting and review, complaints handling, policies in date	5
B2. Theatre and equipment	Theatre condition and air handling, table, lights, diathermy, equipment maintenance and calibration records, instrument sets for the procedures sought	6
B3. Anaesthesia	Anaesthetic machine checks, drugs and controlled drug register, difficult airway kit, pre-anaesthetic assessment, post-anaesthetic recovery criteria	6
B4. Infection prevention	Hand hygiene audit, theatre cleaning schedule, sterile store, surgical site infection surveillance, antibiotic prophylaxis policy	5
B5. Recovery, ward and escalation	Recovery staffing and monitoring, ward nurse ratios, early warning scores, high dependency capacity, transfer drills	6
B6. Patient safety and records	Consent quality, checklist completion in the traced cases, medicines reconciliation, venous thromboembolism assessment, discharge summaries	5
B7. Staffing and credentialing	Rotas covering nights and weekends, staff licence verification, life support training, induction	4
B8. Support services	Laboratory turnaround, blood availability, imaging, pharmacy stock control, water supply	5
B9. Patient experience and outcomes	Information before admission, pain management, outcome recording, readiness to record Mezo outcome measures	4

The maximum score is 92. The score is reported as a percentage, by domain and overall.

PART C: SURGICAL TIERS

A Tier needs both the overall score and every item listed for it.

Tier	Surgery it may host	Overall score	Also required
Tier 1: Day case	Procedures under local, regional or sedation with same-day discharge (for example, minor excisions, cataract, many aesthetic procedures)	60% or more	No domain below 50%. Recovery to discharge criteria.
Tier 2: Inpatient	Planned operations under general anaesthetic with an inpatient stay (for example, hernia, laparoscopic cholecystectomy, myomectomy, breast and body contouring)	70% or more	No domain below 60%. Ward with 24-hour nursing and on-call doctor. Blood available within one hour.
Tier 3: Major and implant	Major, long or implant surgery, and cases that may need intensive care (for example, hip and knee replacement, revision surgery, spine)	80% or more	No domain below 70%. High dependency beds on site and intensive care on site or within thirty minutes by agreement. Blood on site. Ultraclean air or a documented surgical site infection rate for implant work. Image intensifier where the procedures need it.

A facility is approved for a Tier and for a named list of procedures within it. Adding a procedure that needs equipment or skills not seen at assessment needs a focused reassessment.

PART D: KEEPING THE APPROVAL

D1. **Reassessment** every twelve (12) months, and an unannounced visit at least once a year.

D2. **Lapse of a minimum.** If any Part A item lapses, the facility tells Mezo within forty-eight (48) hours and Mezo stops booking surgery there until it is restored.

D3. **Triggered review.** A serious incident, a cluster of complications, or a rising return-to-theatre or infection rate triggers a review within fourteen (14) days, and may move the facility down a Tier.

D4. **Surgeon-owned facilities.** A facility in which a Mezo surgeon has an interest is assessed, reassessed and reviewed by an assessor with no interest in it, to the same criteria. Its outcomes are reviewed by a clinician with no interest in it.

D5. **Transparency to the facility.** The facility receives its full score sheet and the evidence behind any item scored below 2.

PART E: WHAT THE FACILITY PROVIDES BEFORE THE VISIT

Registration certificates (A1, A3, A4); medical director's MDCN licence; insurance certificates; fire certificate; list of theatres, beds and high dependency or intensive care capacity; equipment list with maintenance records; staff list with licence numbers; autoclave cycle and indicator logs for the last three months; the intensive care transfer agreement and blood arrangement; three months of theatre lists; and the procedures for which approval is sought.